

Every Medicaid dollar, verified before it is paid.

A pre-payment fraud tripwire for Medicare and Medicaid, built on public federal data.

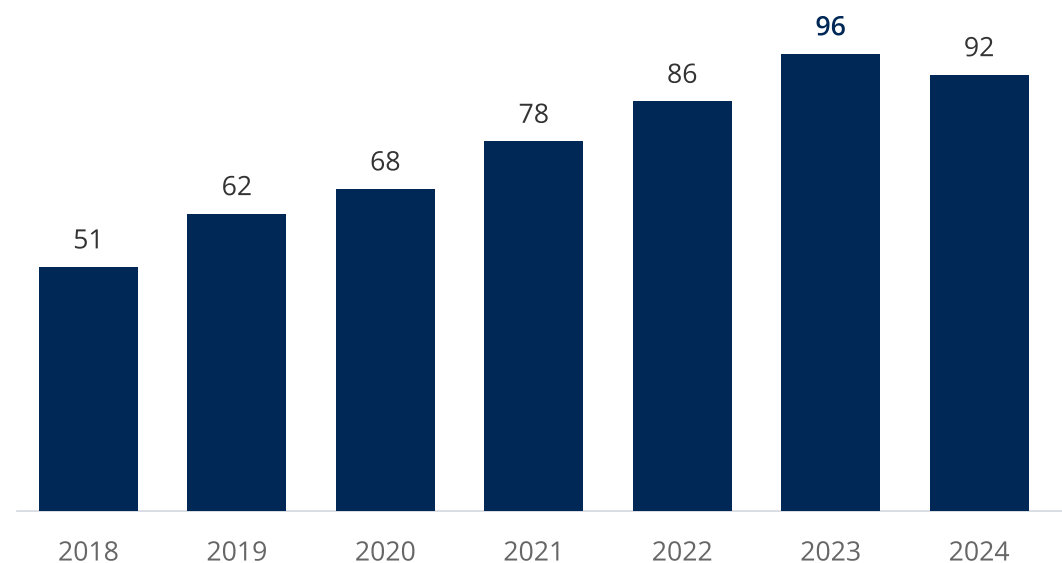
THE PROBLEM

Pay first. Audit later.

Medicaid and Medicare pay claims first and chase the money later. GAO names provider screening as a driving cause.

\$94B

improper payments, FY2025 (GAO-26-108694)



Time-coded Medicaid spending, \$ billions

One warehouse. Three detectors. A cited packet.

1

Ghost networks

Shared owners, suites and phones, resolved into ranked networks.

2

Impossible days

Billed hours checked against the clock.

3

Paid after a screening trigger

Banned providers Medicaid kept paying. Dated and verified.



Every candidate carries its tier.

Five levels of evidence, strongest first. A tier is what the public record proves, not a verdict.

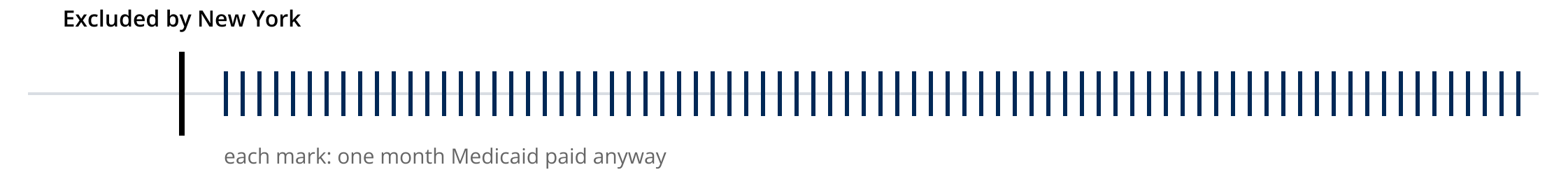
1	On a public list, still paid afterward, or adjudicated in a public enforcement record.	685
2	More hours than a day holds, billed by three or more organizations in the same month.	66
3	In a suspicious network that touches a public list, or charged but not yet adjudicated.	6,517
4	Structure alone: unusual network shape, or impossible hours under a single organization.	2,524
5	Worth knowing, not yet a referral: supervisory volume and informational lists.	2,205

Corroboration outranks volume: two detectors reaching one provider independently is the strongest signal. Dollars count once, from the tier-setting detector.

ONE PROVIDER

Excluded in 2019. Paid for 78 more months.

Every one of those payment months is visible in the public spending file. Verity finds them in seconds.



\$55.9M across 391 providers nationwide, 2018–2024

Candidates for review, not findings.

Conservative by construction.

Impossible days

Units become hours. Untimed codes deliberately under-count.

$$\text{hours}(p, d) = \frac{1}{60} \sum_c u_{p, d, c} \cdot m_c$$

Robust outliers

Medians and MAD, within state and specialty. One giant cannot hide the rest.

$$z = \frac{x - \text{median}(x)}{1.4826 \cdot \text{MAD}(x)}$$

Ranking

Corroboration outranks volume.
Dollars count once, never summed.

$$\text{score} = \text{base}(\text{tier}) + \beta \cdot \text{corroboration} + \gamma \cdot \log(\$)$$

WHAT THE GOVERNMENT SAVES

Three numbers, three strengths of evidence.

\$55.9M

Paid after the strongest-evidence exclusions and revocations, nationwide. Dated, ID-matched, name-verified. Matches OIG's own audited magnitude.

DOCUMENTED

\$1.48B

Paid after a for-cause termination in another state. Ranked with federal corroboration first, offered as a screening queue.

QUEUED

\$532.8B

Time-coded Medicaid spending, 2018–2024, screened continuously instead of audited years later.

VISIBLE

Exposure, never losses. Each dollar counts once.

WHO BUYS

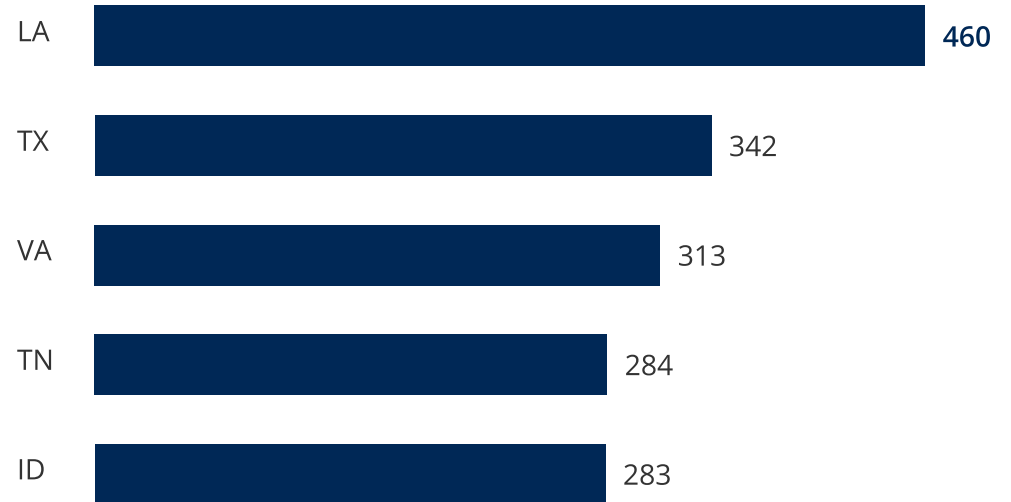
Two buyers. One deadline.

Plans buy first: ghost networks bill their margin, and exclusion penalties hit \$26M in 2025.

States buy bigger: federal law mandates screening infrastructure by January 2027, with funds at risk.

Nobody serves them: incumbents sell to CMS or to hospitals, not to states on a deadline.

“The data is public.
The judgments are ours.”



Revoked or excluded providers still enrolled in Medicaid, by state

V E R I T Y

Stop the next check.

The continuation grant funds one state pilot before the screening deadline. A working integrity layer a program office can defend line by line.